# 6.4 Upper Gastrointestinal Bleeding

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• "I vomited blood and feel faint."<br>**Objective**<br>• Vomiting blood<br>• Black stools (melena)<br>• BP 88/56 mmHg, HR 124/min<br>• Hb 7.5 g/dL | **ACTUAL**<br>**Deficient fluid volume** related to acute blood loss as evidenced by hypotension, tachycardia and low Hb. | Within 24 hours, vital signs stable and Hb on target. | • Put in two large-bore IV cannulas.<br>• Give fluids and blood as prescribed.<br>• Vital signs every 15 minutes until stable.<br>• Nothing by mouth; get him ready for endoscopy.<br>• Give the IV PPI as prescribed.<br>• Record urine output every hour. | Vital signs stable: **goal met**. |
| **Subjective**<br>• "Am I going to die?"<br>**Objective**<br>• Frightened and shaking<br>• Holding tight to a relative's hand | **ACTUAL**<br>**Fear** related to sight of blood and hospital procedures as evidenced by expressed fear. | Says he is less frightened within 2 hours. | • Stay with him.<br>• Explain each procedure briefly and calmly.<br>• Take away anything stained with blood quickly.<br>• Let the family stay. | Calmer: **goal met**. |
| **Subjective**<br>• *No specific complaint*<br>**Objective**<br>• Still bleeding<br>• HR going up | **RISK**<br>**Risk for shock** as evidenced by risk factors: hypovolemia. | MAP kept above 65 mmHg. | • Vital signs every 15–30 minutes.<br>• Watch for signs that he is bleeding again.<br>• Keep blood crossmatched.<br>• Report any change straight away. | No shock: **goal met**. |